



February 18, 2026

The Honorable Robert F. Kennedy, Jr.  
Secretary  
U.S. Department of Health and Human Services  
200 Independence Avenue, SW  
Washington, DC 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy:

The American College of Cardiology (ACC) is the global leader in transforming cardiovascular care and improving heart health for all. As the preeminent source of professional medical education for the entire cardiovascular care team since 1949, ACC credentials cardiovascular professionals in over 140 countries who meet stringent qualifications and leads in the formation of health policy, standards and guidelines. Through its world-renowned family of JACC Journals, NCDR registries, ACC Accreditation Services, global network of Member Sections, CardioSmart patient resources and more, the College is committed to ensuring a world where science, knowledge and innovation optimize patient care and outcomes.

In this request for information (RFI) that follows others by agencies within the Department of Health and Human Services (HHS), HHS seeks public feedback identifying ways regulation, reimbursement, and research & development can accelerate the adoption of AI in clinical care. The ACC appreciates this opportunity to provide feedback and offers comments accordingly.

### **Ensuring Clinician Autonomy**

AI-enabled systems provide well-established benefits by leveraging computing power to enhance patient care. However, the deployment of new tools also comes with great risks that could just as easily put patients in harm's way and threaten the clinician-patient relationship that has been the central tenet of medicine.

Currently the US healthcare system does not have the infrastructure to support autonomous AI. Therefore, the systems established for implementation in the near future are best based in assistive AI or collaborative intelligence, with the clinician as the responsible entity, using AI as a tool. AI systems are inherently trained on large datasets, deriving insights from patterns, averages, and variations across populations. While this approach can uncover meaningful trends, it also risks

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reducing patients to data points rather than individuals with unique contexts and needs. Diagnostic tests or genetic underpinnings where an AI application contributes and enhances patient care are best utilized in coordination with and under the clinical acumen and reasoning of trained clinicians. While the College is acutely aware AI capabilities will continue to evolve, it is essential that any policy frameworks must reaffirm that clinicians—not algorithms—remain at the center of patient care. The current lack of clarity or interpretative guidance from federal agencies on existing policies threatens this autonomy as developers work to deploy systems with the promise of reducing administrative burdens and costs associated with healthcare clinicians and personnel.

The ACC strongly believes that clinical decision making should still lie in the hands of the clinician and the patient, deciding together the best course of action based on what is best for the patient and the evidence available at the time. While AI is a powerful tool, it serves to support clinical decision-making, not replace it. While population-level data remains a foundation, modern AI—through personalization and long context windows—has the unique ability to tailor care based on specific user preferences, social determinants of health (SDOH), and individual goals. Systems established for implementation should support collaborative intelligence. Rather than being limited to population averages, AI can help clinicians uncover unique contexts and needs that might otherwise be overlooked in a traditional clinical setting. Clinicians are essential to interpret, adapt, and individualize these insights to ensure that the art and nuance of medicine are preserved. AI's role is to enhance care while preventing unnecessary treatment. The starting point for such decisions must always be the individual patient. The College believes AI can play an active role in helping clinicians explore options available to a patient by presenting information and serving as a tool adjunctive to clinician decision making, but it is important to ensure that clinician autonomy is not overridden, and software does not supplant the clinician-patient relationship.

### **Evolving Standards of Care and Liability**

It is important that any policy framework should clearly and unequivocally ensure that AI/ML-driven solutions are not considered or presumed as the new “standard of care” in any way. There is great uncertainty on the potential risks and liability a clinician faces should they deviate from recommended AI-enabled software outputs, whether autonomous or augmentative. While many government agencies may not establish liability criteria, reiterating that these are simply tools that are used to inform the decision-making process, rather than dictating it, will help ensure that clinicians remain in charge of a patient’s care. By taking these actions, the government can clarify the role of clinicians and tools such as AI to maintain the needed trust in the healthcare system.

For those health AI tools specifically designed and validated to be autonomous, industry precedents demonstrate that autonomous AI can safely operate within the healthcare ecosystem; for instance, companies like IDx-DR (LumineticsCore, for diabetic retinopathy detection) and Doctrone (for automated prescription refill pilot in Utah) have successfully utilized liability insurance to protect against harm. The government should encourage a framework where developers can assume liability for specific medical diagnosis, treatment, and management use cases, thereby lowering the barrier for clinical adoption.

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## Privacy and Cybersecurity

While federal agencies, including those HHS, have made progress to advance cybersecurity in healthcare, it is important that agencies build on this momentum and develop a clear and consistent baseline for data privacy and cybersecurity in healthcare AI. Agencies must develop clear regulations, frameworks and guidance that place these obligations on the entities that build and deploy AI tools, not just on individual clinicians. This includes clear guidance to regulated entities on the privacy and cybersecurity requirements for AI tools and suggested best practices for non-regulated developers, aligned with one another to ensure consistency across the healthcare system.

As technology rapidly advances, existing privacy and cybersecurity regulations and guidance, including those like HIPAA overseen by the Office for Civil Rights (OCR) or frameworks developed by the National Institute of Standards and Technology (NIST), must account for new age technology and threats. The advent of quantum computing and advanced computational techniques threatens to nullify existing privacy and security techniques required or suggested by federal agencies. Therefore, it is essential that these agencies develop policies that adequately ensure that information, often highly sensitive in nature, can be sufficiently protected.

## Flexibility

It is important that regulatory agencies draft policies and guidance that are flexible enough to distinguish between AI-enabled health care applications that actively participate in the practice of medicine, either defined by the Federal Food, Drug, and Cosmetic Act (FD&C Act) and the 21st Century Cures Act, and more broadly designed AI systems that assist with practice of medicine through administrative simplification or other essential tasks. Future AI systems may blur the line between regulated and non-regulated activities. Any actions should allow agencies sufficient regulatory flexibility to provide clear guidance to clinicians, developers, and stakeholders, so they are clear on what is and is not regulated. Regulatory ambiguity hampers innovation, promotes distrust in systems, and results in the development of fewer applications of this promising technology. Federal agencies should ensure that guidance is constantly reassessed to account for iterative changes in technologies and provide the clearest regulatory picture possible.

## Unified Approach to Governance

Any federal action to address statutes, regulations, or policies that govern the use of AI in healthcare should ensure sufficient coordination across the HHS and the Federal Trade Commission (FTC), with HHS leadership aligning its agencies—Food and Drug Administration (FDA), Centers for Medicare & Medicaid Services (CMS), Assistant Secretary for Technology Policy/Office of the National Coordinator for Health IT (ASTP/ONC), and OCR. A unified AI governance framework should establish clear standards on transparency, model card development, risk stratification to prevent overregulation, funding for comparative effectiveness research, harmonized definitions across federal agencies, and real-world case studies to guide clinicians and patients.

Many of these concepts already exist and are defined either through regulations or sub-regulatory guidance in other components of the US health care system and are accepted by developers, researchers, and experts from across the ideological spectrum. Borrowing from and building on existing concepts and developing a unified, whole of government approach can remove subjectivity

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from the decision-making process, boost adoption, and promote the development of innovative AI systems that are cutting edge, transparent, and accurate.

At the same time, the growing patchwork of state-based AI policies poses challenges for consistent innovation and implementation. Divergent requirements across jurisdictions can create uncertainty for developers, increase administrative complexity for clinicians, and delay the safe and equitable deployment of AI tools to patients. Ensuring alignment and interoperability of frameworks without constraining legitimate state flexibility will be essential to fostering innovation while maintaining trust, safety, and access to high-quality care.

### **Establishing Public Trust in AI**

Tangential to the needed unified approach to governance is the need for all agencies to work together to establish and maintain public trust in AI. Federal agencies operate with defined scopes and missions, yet concepts like trust are often ignored as it is hard to develop policies that achieve the aim of an ambiguous term like trust. This is why it is even more essential that all interested parties, including developers, agencies, private sector organizations, and other interested parties work together to ensure that the public can trust that AI will be used in a safe, transparent, and accurate way.

One of the prevailing ways to address this is through the reduction of AI bias in outputs. IBM refers to AI-bias, also called machine learning bias or algorithm bias, as the occurrence of biased results due to human biases that skew the original training data or AI algorithm, leading to distorted outputs and potentially harmful outcomes. In medicine, these biases can lead to inaccurate diagnosis and result in patient harm or death.

Due to the nature of health care, it is essential that any policy actions work to ensure that any potential biases in data inputs, AI-enabled outputs, and passive and automated systems are minimized as much as possible or adjusted for accordingly to ensure that outputs do not lead to biased or incorrect outputs. Many of the top AI developers are actively working on methods to address and reduce bias, ensuring that systems produce accurate results, and the College believes that any action should consider this important work and reflect the importance of this issue.

To that end, it is important that actions work to instill trust by the public through the development of policies that promote transparent, clinically validated AI systems in health care. This includes providing patients, clinicians, and other stakeholders with a firm understanding of and information about data sets used to train AI models. Successful AI-enabled technologies require early engagement from clinicians and organizations like the ACC that stand at the nexus of this innovation. The ACC strongly recommends any actions taken should eliminate barriers that prevent the adoption of trust-based AI and promote policies that encourage AI system developers to integrate clinicians' perspectives into the design, development, validation, implementation, and monitoring of AI in health care.

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## Communication to the Public

Transparency and clear communication are vital to maintaining public trust in AI systems. The ACC appreciates that traditional regulatory mechanisms may not always serve as the most appropriate method for communicating with the public and updating guidance in an environment with rapid innovation and iteration. Agencies should regularly and clearly communicate with stakeholders through numerous mechanisms, including utilizing traditional rulemaking processes. The rulemaking process affords the public the ability to clearly see comments submitted by other stakeholders and affords regulatory agencies the ability to respond to insights and concerns in writing. These responses often provide the clearest understanding of regulatory agencies' decision-making processes and helps the public comprehend why a decision was made. The ACC calls on agencies to remain transparent, clearly promote any guidance and rulemaking documents, and regularly communicate with the public to ensure trust is maintained.

## Conclusion

The ACC appreciates the opportunity to provide input on the Request for Information on Regulatory Reform on Artificial Intelligence. If you have any questions or follow up, please contact James Vavricek, Director of Regulatory Affairs at [jvavricek@acc.org](mailto:jvavricek@acc.org).

Sincerely,



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